



BEHAVIORAL EMERGENCY - COMBATIVE

Effective: January 1, 2026
Replaces: January 1, 2025

1. Patient Care Goals

- 1.1. Provision of emergency medical care to the agitated, violent, or uncooperative patient
- 1.2. Maximizing and maintaining safety for the patient, EMS personnel, and others

2. BLS Treatment

- 2.1. Routine Medical Care – Adult (700-S04)
 - 2.1.1. **Oxygen** – titrate as appropriate if possible
- 2.2. Request law enforcement assistance if not already on scene
- 2.3. If there are enough responders on scene, attempt to safely restrain the patient (700-S13)
- 2.4. **Blood Glucose Level**, to rule out hypoglycemia, if possible
 - 2.4.1. If hypoglycemic see (700-A03)

3. ALS Treatment

- 3.1. **Pulse Oximetry**, to rule out hypoxia, if possible
 - 3.1.1. If hypoxic, **Oxygen** – high flow

4. Behavioral Sedation

- 4.1. The ONLY prescribed use of sedation is to reduce the emergency medical condition of agitated or combative behavior that endangers the patient and/or the healthcare providers, when de-escalation efforts are unsuccessful
- 4.2. To be eligible for sedation the patient must also fit all the following criteria:
 - 4.2.1. 15 years of age or older
 - 4.2.2. Less than 65 years of age
 - 4.2.3. Patient does not have a suspected traumatic head injury or stroke
 - 4.2.4. Patient is presumed to not have a medical condition causing the agitation such as hypoglycemia or hypoxia
 - 4.2.5. EMS Sedation is **not** indicated to facilitate law enforcement activities
- 4.3. Administer **Midazolam 5 mg IM**, may repeat once, if agitation and combative behavior continue after ten minutes
 - 4.3.1. If midazolam is administered, patient must be transported to an emergency department and maintain continuous airway monitoring
- 4.4. As soon as patient's condition allows:
 - 4.4.1. Assess airway patency and respirations, suction as needed
 - 4.4.2. Restrain in a supine or head slightly elevated position (700-S13), **never place patient in a prone position**
 - 4.4.3. **Oxygen** – titrate as appropriate
 - 4.4.4. **Pulse Oximetry**
 - 4.4.5. **Capnography**
 - 4.4.6. **Vascular Access (IV) TKO**



5. Severe Agitation with Altered Mental Status

- 5.1. Patients meeting all the following criteria should be considered candidates for treatment of Severe Agitation with Altered Mental Status. Immediate **BASE CONTACT** should be made for midazolam.
 - 5.1.1. 15 years of age or older
 - 5.1.2. Less than 65 years of age
 - 5.1.3. Altered mental status
 - 5.1.4. Patient does not have a suspected traumatic head injury or stroke
 - 5.1.5. Extreme agitation
 - 5.1.6. Pain tolerance
 - 5.1.7. Tactile hyperthermia
 - 5.1.8. Physical destructiveness
 - 5.1.9. De-escalation methods fail
 - 5.1.10. Violence toward inanimate objects
 - 5.1.11. Endurance without apparent fatigue, unstoppable
- 5.2. **BASE CONTACT: Midazolam 10 mg IM**
- 5.3. Upon pharmacological control of the patient's behavior, immediately
 - 5.3.1. Assess airway patency and respirations, suction as needed
 - 5.3.2. Restrain in a supine or head slightly elevated position **(700-S13)**, never place patient in a prone position
 - 5.3.3. **Oxygen** – titrate as appropriate
 - 5.3.4. **Pulse oximetry**
 - 5.3.5. **Capnography**
 - 5.3.6. **Vascular Access (IV) TKO**
 - 5.3.7. Active cooling as needed
 - 5.3.8. Transport to closest most appropriate emergency department

6. Pertinent Assessment Findings

- 6.1. Continuous monitoring of:
 - 6.1.1. Airway Patency
 - 6.1.2. Respiratory status with pulse oximetry and capnography
 - 6.1.3. Circulatory status with frequent blood pressure measurements
 - 6.1.4. Mental status and trends in level of patient cooperation
 - 6.1.5. Cardiac status, especially if the patient has received pharmacologic management medication
 - 6.1.6. Extremity perfusion with capillary refill in patients in physical management device

7. Key Documentation Elements

- 7.1. Etiology of agitated or violent behavior if known
- 7.2. Patient's medications, other medications or substances found on scene
- 7.3. Patient's medical history or other historic factors reported by patient, family, or bystanders
- 7.4. Physical evidence or history of trauma
- 7.5. Adequate oxygenation by pulse oximetry
- 7.6. Blood glucose measurement
- 7.7. Dose, route, and number of doses of pharmacologic management medications administered and response to Pharmacological management
- 7.8. Number and physical sites of placement of physical management devices



- 7.9. Duration of placement of physical management devices
- 7.10. Repeated assessment of airway patency
- 7.11. Repeated assessment of respiratory rate, effort, pulse oximetry/capnography
- 7.12. Repeated assessment of circulatory status with blood pressure, capillary refill, cardiac monitoring
- 7.13. Repeated assessment of mental status and trends in the level of patient cooperation
- 7.14. Repeated assessment of capillary refill in patient with extremity securing devices
- 7.15. Base Hospital contact
- 7.16. Initiation and duration of engagement with law enforcement